

Dermatoscopic findings

They consist of white crossing lines (Wickham sign), dull red background, and peripheral arrangement of vessels.

Other assays

It has been proposed that future diagnostic adjuncts could include cytokine profiling of involved tissue. However, as other available tests are reasonably accurate for the diagnosis of LP, this is unlikely to become a widely used assay for LP.

Patch tests in patients with LP usually reveal positive results, which are not associated with specific variants of cutaneous LP or the prognosis of the disease. Patch testing is better to be included in the workup of oral LP, where oral lesions show proximity to dental restorations. The material of these restorations can aggravate or induce the lichenoid reaction.

No imaging studies are necessary for the diagnosis of lichen planus.

Differential diagnosis

There are many disorders to be considered in the differential diagnosis of lichen planus:

- Lichen nitidus, lichen sclerosus, lichen spinulosus, graft-versus-host disease, lichen striatus, linear epidermal naevus, naevus unius lateralis
- Eczema, lichen simplex chronicus, prurigo nodularis
- Pityriasis rosea, guttate psoriasis, psoriasis vulgaris, eczematid-like purpura
- Drug eruption, syphilis, tinea corporis, papular acrodermatitis of childhood

- Granuloma annulare, lichen amyloidosis, pityriasis lichenoides, Kaposi sarcoma

Clinical variants of lichen planus

Several variations have been described, according to (i) the distribution and configuration of lesions, (ii) the morphology of an individual lesion, and/or (iii) the site of involvement. The various clinical forms are divided into three general categories: cutaneous, appendageal, and mucosal lichen planus, as well as some unclassified other forms of LP (Table 2). More information about the clinical variants is available in the longer version on the EDF homepage.

Complications

Acute complications

Itching is most common in cutaneous LP. In oral LP, eating might be so painful that patients are unable to maintain adequate nutrition. Oral lichen planus and the drugs used for treatment may predispose to infection from *C. albicans*. Sexual dysfunction may also occur and can become a long-term complication in genital erosive lichen planus.

Chronic complications

After resolution of lichen planus lesions, the affected skin area often develops postinflammatory hyperpigmentation, which is more noticeable in individuals with darker skin.